"The estimated age-standardized prevalence of diabetes

Generated
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Report version
2

Executive finding

The submitted dataset permits a reproducible descriptive comparison. See the exact observations and scope below.
Scientific claim assessment remains not evaluated. Model review and software execution are separately reported.

Reported values and calculations

Diabetes prevalence (% of population ages 20 to 79) · 2024
Egypt: 22.4 (row 124)
United States: 13.7 (row 434)
Difference: 8.7 percentage points
Ratio: 1.635036496350364963503649635036496
Decimal subtraction at precision 64; first-value / second-value ratio at precision 34 with half-even rounding. Inputs limited to 28 digits and exponent ±28. No significance test or causal inference.
Reported values in the same submitted indicator and year. Population definitions, age-standardization, methodology and source authenticity have not been independently established.

Diabetes prevalence (% of population ages 20 to 79) 2024 · Reported observations

Egypt higher than United States



Difference (first − second)

8.7 percentage points

Ratio (first ÷ second)

1.635

Exact observations and method

2024 · %

Observation	Value	Source record row
Egypt	22.4	124
United States	13.7	434

Decimal subtraction at precision 64; first-value / second-value ratio at precision 34 with half-even rounding. Inputs limited to 28 digits and exponent ±28. No significance test or causal inference.

Exact ratio 1.6350364963503649635036496350364963

Linear display of original values with a zero baseline. Plot version reported-comparison/1

Reported values in the same submitted indicator and year. Population definitions, age-standardization, methodology and source authenticity have not been independently established.

Original claims

Investigate the following scientific claim:

"The estimated age-standardized prevalence of diabetes among adults aged 20–79 was higher in Egypt than in the United States in 2024."

Using the attached dataset and source, please:

1. Compare the reported diabetes prevalence percentages for Egypt and the United States.
2. Identify the relevant year, population, units, and indicator definition.
3. Check whether the available numerical data support the claim.
4. Examine the reliability and limitations of the evidence.
5. Distinguish observed differences from possible causal explanations.
6. Identify any missing evidence or verification steps.
7. Explain what conclusions can and cannot be drawn.

Do not assume that a difference in diabetes prevalence establishes its causes.

Clearly distinguish independently performed calculations, AI interpretations, and checks that could not be completed.

Submitted evidence

diabetes-prevalence.csv — text/csv
diabetes-prevalence.png — image/png

Source and provenance

No external source was retrieved.

Checks performed and unavailable

Structured data inspection Parsed tables are available for bounded queries. Inspect the selected rows and their provenance.	Performed
Reported value comparison Exact observations and arithmetic are recorded above.	Performed
Source reading No supported URL was recorded for this run. New reviews also detect links in questions and research notes.	No supported source link
Independent source authentication Independent source authentication is unavailable in this pipeline. Source provenance remains unverified.	Not performed
Statistical inference Validated inferential procedures are unavailable. Descriptive calculations are reported separately.	Not performed
Blind plot reconstruction Blind reconstruction awaits a specified and accepted blinding/rendering protocol. Original charts remain inspectable.	Unavailable

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Visual Analyst — AI interpretation

The visualization is a choropleth map that generally represents the provided data trends, but it utilizes a non-linear color scale that may exaggerate differences between lower prevalence categories while compressing higher ones. The visual encoding for Egypt and

the United States is consistent with the provided dataset values.
Non-linear Color Scale Intervals: The legend intervals are inconsistent: 0%, 2%, 4%, 6%, 8%, 10%, 12.5%, 15%, 17.5%, 20%. The intervals start at 2 percentage points and then shift to 2.5 percentage points after 10%.
Ambiguous Category Boundaries: The legend provides point values (e.g., 10%, 12.5%) rather than explicit ranges (e.g., 10%–12.5%).

Limitations and next steps

Source authenticity and methodological comparability have not been independently established.
Inspect the source definitions, population, units and methods before extending the descriptive finding.

Council outputs are AI interpretations. Shared models may share errors; agreement does not establish scientific truth.
Inspect cited observations, calculations and unresolved disagreements independently.

Independent blind plot reconstruction is unavailable. Its blinding and rendering protocol still needs specification and acceptance.
Use the descriptive data plot and original image separately. Neither is a validated neutral reconstruction.

Reproducibility

Input hashes, actual model names, structured outputs and execution events are retained.
Same-provider isolated calls do not establish independent scientific confirmation.

Reproducibility information

Evidence processing descriptive-evidence/1. See exact row references, source snapshots, prompt snapshots, per-role inputs and raw outputs in the evidence and audit record.

PDF uses your browser's print dialog. Original records remain available separately from machine translations.

Evidence limitations remain part of the assessment. Agreement between models is not scientific proof.